

ADVANCED NURSING QUESTIONS

1. Which of the following is not a common indication for oxygen therapy?
 - A. Hypoxemia
 - B. Chronic obstructive pulmonary disease (COPD)
 - C. Asthma exacerbation
 - D. Hypertension

2. Which of the following is an appropriate nursing intervention during oxygen therapy?
 - A. Administering oxygen at a flow rate of 10 L/min via a nasal cannula
 - B. Ensuring the oxygen tubing is kinked to prevent oxygen flow
 - C. Monitoring oxygen saturation levels using a pulse oximeter
 - D. Encouraging the patient to smoke while on oxygen therapy

3. Which of the following is a potential complication to oxygen therapy?
- A. Hypoxemia
 - B. Hypercapnia
 - C. Hypotension
 - D. Hyperventilation
4. When providing oxygen therapy, the nurse should prioritize which of the following?
- A. Ensuring the oxygen source is located near a heat source
 - B. Administering oxygen at the highest possible flow rate
 - C. Assessing the patient's respiratory status regularly
 - D. Allowing the patient to adjust the oxygen flow rate as desired
5. As an advanced student nurse who is preparing for your licensure exams, what do you think is the purpose of a retweather mask, in oxygen therapy?
- A. To deliver a high concemration of oxygen

- B. To humidify the oxygen before inhalation
 - C. To provide a comfortable fit for the patient
 - D. To prevent the rebreathing of exhaled carbon dioxide
6. As an advanced student nurse who is preparing for your licensure exams, how should the junior nurse ensure a proper fit of the rebreather mask?
- A. Secure the mask tightly to the patient's face
 - B. Adjust the straps to ensure a snug fit without discomfort
 - C. Allow the patient to adjust the mask as desired
 - D. Use a larger-sized mask to ensure better oxygen delivery
7. Which of the following should the nurse assess first when a patient is using a rebreather mask for oxygen therapy?
- A. Oxygen flow rate and temperature
 - B. Skin integrity around the mask area
 - C. Respiratory rate and effort

D. Blood pressure and heart rate

8. Nurse manager Esther Agymang is educating junior staff on oxygen administration, how often should the nurse monitor the patient's oxygen saturation levels during rebreather mask therapy

A. Every 2 hours

B. Every 4 hours

C. Every 6 hours

D. Every 8 hours

9. Which of the following is the most appropriate initial nursing action before initiating a blood transfusion?

A. Checking the patient's identification

B. Assessing the patient's vital signs

C. Verifying the blood type and crossmatch

D. Obtaining informed consent from the patient

10. During blood transfusion, the nurse should monitor the patient closely for which of the following as a common signs or symptoms of a transfusion reaction

- A. Hypertension and tachycardia
- B. Flushed skin and itching
- C. Increased urine output and diaphoresis
- D. Decreased respiratory rate and blood pressure

11. Midwife Yayera understands that, in the process of blood transfusion and compatibility crisis, the most appropriate nursing action if a transfusion reaction is suspected will be

- A. Stop the transfusion immediately and notify the healthcare provider
- B Increase the transfusion rate to flush out the reaction
- B. Administer antihistamines and continue the transfusion
- C. Document the reaction and continue monitoring the patient

12. Nurse Anamdare understands that observation and monitoring are paramount in blood transfusion. How long should it take to administer a unit of packed red blood cells during a blood transfusion?

A. 1 hour

8. 2 hours

B. 4 hours

C. 6 hours

13. After a successful course in advance nursing this semester, nurse Mrs. Olivia Arthur's take home point in bladder catheterization included the knowledge that, the most common indication for bladder catheterization is?

A. Urinary incontinence

B. Urinary tract infection

C. Bladder cancer

D. Kidney stones

14. Nurse Rhoda's take-home point in bladder catheterization included the knowledge that, the most appropriate nursing intervention during bladder catheterization?

- A. Using sterile technique during catheter insertion
- B. Emptying the drainage bag once a week
- C. Securing the catheter loosely to the patient's leg
- D. Removing the catheter as soon as the patient requests it

15. Which of the following is a potential complication of bladder catheterization?

- A. Increased urine output
- B. Urinary retention
- C. Decreased risk of urinary tract infection
- D. Improved bladder control

16. When providing nursing care for a patient with a bladder catheter, the nurse should prioritize which of the following?

- A. Ensuring the catheter bag is positioned below the level of the bladder
- B. Changing the catheter every day to prevent infection

C. Encouraging the patient to drink minimal fluids to reduce urine output

D. Allowing the patient to ambulate Geely without considering the catheter

A. Placenta previa

17. Which of the following is not a common cause of antepartum hemorrhage?

B. Placental abruption

C. Uterine rupture

D. Ectopic pregnancy

18. What is the priority nursing action when caring for a patient with antepartum hemorrhage?

A. Administering pain medication

B. Monitoring fetal heart rate

C. Assisting with vaginal exams

D. Providing emotional support

19. Which position is recommended for a patient with antepartum hemorrhage?

- A. Supine position
- B. Trendelenburg position
- C. Left lateral position
- D. Sitting upright position

20. What is the appropriate nursing intervention for a patient with suspected placenta previa and is actively bleeding?

- A. Encouraging ambulation to promote blood flow
- B. Administering oxytocin to stimulate uterine contractions
- C. Preparing for an immediate cesarean section
- D. Applying pressure to the umbilical cord

C

- A. Uterine many

21. Which of the following is the most common cause of postpartum hemorrhage?

B. Placental abruption

C. Uterine rupture

D. Retained placenta

22. What is the first nursing action when managing postpartum hemorrhage?

A Administering uterotonic medications

B Assessing vital signs

C. Massaging the uterus

E. Notifying the healthcare provider

23. Which of the following is a sign of internal bleeding during postpartum hemorrhage?

A. Blood pressure elevation

B. Decreased heart rate

C. Pale and clammy skin

D. Increased urine output

24. From the list of steps/ measures provided, which can be adapted by the intraoperative scrub nurse as the most efficient method to assist in preventing surgical site infections

A. Ensuring proper sterilization of surgical names

H. Maintaining a clean and sterile environment

B. Administering prophylactic antibiotics as ordered

C. All of the above

25. Which patient position is commonly used for abdominal surgery

A. Supine position

B. Trendelenburg position

C. Lithotomy position

D. Prone position

A

26. Which patient position is commonly used for surgeries involving the head and neck?

- A. Supine position
- B. Trendelenburg position
- C. Lithotomy position
- D. Prone position

27. Which patient position is commonly used for surgeries involving the anal or rectal regions?

Regions?

- A. Supine position
- B. Trendelenburg position
- C. Lithotomy position
- D. Prone position

28. Which patient position is commonly used for surgeries involving the chest or thoracic regions?

Region?

- A. Supine position
- B. Trendelenburg position
- C. Lithotomy position
- D. Prone position

29. Which patient position is commonly used for surgeries involving the pelvis or lower

Abdomen?

- A. Supine position
- B. Trendelenburg position
- C. Lithotomy position
- D. Prone position

30. Which of the following underlisted instrument is commonly used to grasp and hold tissues during surgery?

A Dissecting Forceps

B. Scalpel

C. Retractor

E. Artery forceps

31. Which of the following instrument is used to cut through tissues during

Surgery? A. Dissecting Forceps

B. Scalpel

C. Retractor

D. Artery forceps

A Forceps

32. Which of the following instrument is used to hold back tissues of organs to provide better visualization during vaginal examinations?

B. Scalpel

C. Cascos

D. Hemostat

C

33. Which of the following underlisted instrument is used to clamp blood vessels to control bleeding during surgery?

A. Dissecting Forceps

B. Scalpel

C. Retractor

D. Artery forceps

34. Which of the following underinted instrument is commonly used to cut and dissect issues during surgery?

A. Surgeon Scissors

B. Needle holder.

C. Suture material

D. Trocae

35. Which of the following underlisted instrument is used to hold and manipulate needles during suturing

A. Stitch Scissors

B. Needle holder

C. Suture material

D. Trocar

36. Which of the following underlisted instruments is used to close surgical incisions or

Wounds?

A. Stitch Scissors

A. Needle holder

B. Suture material

C. Trocar

37. Which of the following underlined instruments is used to create an opening in a body cavity during, laparoscopic surgery?

A. Surgeon Scissors

B. Needle holder

C. Suture material

D. Torcar

38. Which of the following is most commonly used to remove foreign objects or to take samples from the wound?

A. Curette

B. Retractor

D. Hemostat

E. Trocar

39. Which of the following is the instrument used to hold and manipulate tissues during delicate procedures?

A. Rongeur

B. Retractor

C. Hemostas

D. Trocar

40. Which of the following underfisted instrument is commonly used to remove sutures ar staples from surgical incisions?

A. Suture scissors

B. Retractor

C. Hemostat

D. Trocar

41. Which of the following underlisted instrument is used to hold and manipulate the uterus during gynecological procedures

A. Uterine sound

B. Retractor

C. Sim's speculum

D. Trocar

42. Which of the following underlisted instrument is commonly used to remove polyps or abnormal tissue during endoscopic procedures

- A. Snare
- B. Retractor
- C. Sim's speculum
- D. Trocar

43. Which of the following underlisted instrument is used to dilate or enlarge an opening or passage during procedures like manual vaginal evacuation?

- A. Dilator
- B. Retractor
- C. Sim's Speculum
- D. Trocar

44. The client has a prescription for a calcium carbonate compound to neutralize stomach acid. The nurse should assess the client for:

- A. Constipation

B. Hyperphosphatemia

C. Hypomagnesemia

D. Diarrhoea

45. A nurse is assessing a client hospitalized with peptic ulcer disease. Which finding should be reported to the charge nurse immediately?

A. BP 82/60, pulse 120

B. Pulse 68, respirations 24

C. BP 110/88, pulse 56

D. Pulse 82, respirations 16

46. Removal of devitalized tissue in pressure ulcers when appropriate for the resident's condition and consistent with resident goals is

A. Irrigation

B. Sterile technique

C. Debridement

D. Exudates

47. These are surgical wounds in which the respiratory, alimentary, genital, or urinary tract has been entered

A. Clean wounds

B. Clean-contaminated wounds

C. Contaminated wounds

D. Dirty or infected wounds

48. These wounds that are left open for 3-5 days to allow edema, infection, or exudate to drain. This type of wound healing can be referred to as

A. Primary healing

B. Secondary healing

C. Tertiary healing

D. Quaternary healing

49. This phase of healing extends from day 3 or 4 until day 21 following injury.
Collagen Increases in the area, capillaries grow across the wound.

A. Inflammatory phase

B Proliferative phase

B. Maturation phase

C. Bleeding phase

50. When assessing a lesion diagnosed as malignant melanoma, the nurse in charge most likely expects to find which of the following?

A. An irregular shaped lesion

B. A small papule with a dry, rough scale

C. A firm, nodular lesion topped with crust

C. A pearly papule with a central crater and a waxy border

51. Surgical wound infection can be caused by:

A. The environment

B. The instruments used for the operations

C. The staff working in the operating theatre

D. All the above

52. To promote healing of a large incision, a patient should be given which of these vitamins?

A. Amino acid

B. Ascorbic acid

C. Calcium

D. Thiamine

53. Madam Amina undergone exploratory laparostomy and she is on your ward. During your daily wound dressing she suddenly coughs resulting in a burst abdomen. What will be your immediate action?

A. Cover the wound with sterile towel and report to your in charge

B. Irrigate the exposed area with sterile water or normal saline and report to your in charge

C. Pack the intestines back to the abdomen and report to your in charge

D. Screen the bed immediately and report to the in charge

54. A client scheduled for an exploratory laparotomy tells the nurse that she takes kava-kava (piper methysticum) for sleep. The nurse should notify the doctor because kava-kava:

A. Increases the effects of anesthesia and post-operative analgesia

B. Eliminates the need for antimicrobial therapy following surgery

C. Increases urinary output, so a urinary catheter will be needed post-operatively

D. Depresses the immune system, so infection is more of a problem

55. The nurse is teaching circumcision care to the mother of a newborn. Which statement indicates that the mother needs further teaching?

A. "I will apply a petroleum ointment to the area once a day

B. "I will clean the area carefully with each diaper change."

C. I can place a heat lamp next to the area to speed up the healing process

D "I should carefully observe the area for signs of infection."

56. The nurse in caring for a client following helolithotomy Postoperatively, the client should be positioned

A. On the right side

B. Supine

C. On the left side

D. Prone

57. One (1) unit of blood often transfused, will raise patient Hemoglobin level by how much?

A. 0.25-0.75g/dL

B. 0.75-1.2g/dL

C 1-1.5g/dL

C. None of the above

58. What are the common risks involved in donating blood

A Bacterial infection

B. Low blood pressure

C. Contact common viruses

D. None of the above

59. As a therapeutic procedure, transfusions are undertaken on a daily basis within health care facilities. The following are reasons for which transfusions are given to a person EXCEPT?

A. To increase the blood's ability to carry oxygen.

B. To increase the amount of blood.

C. To decrease the risk of bleeding.

D. None of the above

60. The nurse is preparing a client with gastroesophageal reflux disease (GERD) for drainage. The nurse should tell the client to

A. Eat a small snack before bedtime

B. Sleep on this right side

C. Avoid coke, tea, and coffee

D. Increase his intake of citrus fruits

61. The man is preparing to walk the post-operative client for the first time since surgery. Before walking the client, their never should?

A. Give the client pain medication B. Assist the client in dangling his legs

B. Have the client breathe deeply

C. Provide the client with additional fluids

62. A client admitted for treatment of duodenal ulcer complains of sudden sharp mid epigastric pain. Further assessment reveals that the he has a rigid, boardlike abdomen. The nurse recognizes that the client's symptoms most likely indicate

A. Ulcer perforation

B. Increased ulcer formation

C. Esophageal inflammation.

D. Intestinal obstruction

63. A post-operative client has called the nurse's station with complaints of pain. The first action by the nurse should be to

- A. Check to see when the client received pain medication
- B. Administer the prescribed pain medication
- C. Notify the charge nurse of the client's complaints
- D. Assess the location and character of the client's pain

64. The physician has ordered a straight catheterization for a female client. When performing a straight catheterization on a female client, the nurse should

- A. Use medical asepsis when doing the catheterization
- B. Insert the catheter 4-6 inches
- C. Inflate and deflate the balloon before insertion
- D. Hold the catheter in place while the bladder empties

65. Before moving a client up in bed, the nurse lowers the head of the bed. The purpose of lowering the head of the bed is:

- A. To avoid working against gravity as the client is moved
- B. To prevent getting wrinkles in the client's linen

C. To eliminate needing additional help to move the client

D. To relieve pressure on the client's sacrum

66. The nurse is caring for a client with esophageal cancer. The client's history will likely reveal:

A. A diet high in fiber

B. Presence of gastroesophageal reflux

C. Occasional use of alcohol

D. A diet low in fat

67 conditions or disorders are treated surgically, or medically/pharmacologically

Is the nursing care of adult patients/clients whose

A. Surgical Nursing

B. Advanced Nursing 1

C. Pharmacological Nursing

D. Surgical-Medical Nursing

67. Which of the following type of surgery can be referred to as merely symptomatic operation

A Ablative surgery

B. Constructive surgery

C. Exploratory surgery

E. Palliative surgery

68. The following are examples of classification of surgery according to urgency see

A. Elective surgery

B. Planned surgery

C. Premeditated

D. Major surgery

69. The medical term or diagnosis involving traumatic blood loss to the extent that, clotting properties are all lost and the only means of resuscitation is blood transfusion is known

A. Disseminated intravascular coagulation

8. Distractive intravenous courgulation

B. Cardiopulmonary resuscitation

C. All the above

70. The nurse is positioning a client with right hemiplegia. To prevent subluxation of the client's right shoulder, the nurse should

A. Use a pillow to support the client's arm when she is sitting in a chair

B. Elevate the arm and hand above chest level when she is lying in bed

C. Place a pillow under the axilla to elevate the elbow when she is lying in bed

D. Use a pillow to support the client's hand when she is sitting in a chair

71. A client is receiving external radiation for cancer of the larynx. As a result of the treatment, the client will most likely complain of

A. Generalized pruritis

B. Dyspnea

C. Sore throat

D. Bone pain

72. Which one of the following measures decreases abdominal discomfort when the postoperative client is asked to cough

A. Exhaling forcefully between coughs

B. Splinting the incision with a pillow

C. Maintaining muscle tension in the operative site

D. Taking panting respirations between coughs

73. An infant born at 25 weeks gestation was treated with prolonged oxygen therapy. Prolonged oxygen therapy places the infant at risk for

A. Cerebral palsy

B. Retinitis pigmentosa

C. Hydrocephalus

D. Retinopathy of prematurity

74. The nurse should assess a client who has a peptic ulcer for signs of bleeding. Which symptom would best indicate this complication?

A. Melena

B. Hematuria

C. Hemoptysis

D. Ecchymosis

75. A 45-year-old client returned from a colon resection 2 hours ago. Which vital signs indicate possible hemorrhagic shock?

A. BP 120/80, heart rate 88

B. BP 170/100, heart rate 120

C. BP 160/98, heart rate 54

D. BP 96/60, heart rate 120

76. The nurse is performing a self-breast exam when she discovers a mass. Which characteristic of the mass would be most indicative of a malignancy?

A. Tender to touch

B. Regular shape

C. Moves easily

D. Firm to the touch

77. The nurse is assessing a client after a car accident. Partial airway obstruction is suspected. Which clinical manifestation is a late sign of airway obstruction?

A. Rales auscultated in both lungs

B. Restlessness

C. Cyanotic ear lobes

D. Inspiratory stridor

78. Which exam is most reliable in diagnosing peptic ulcer disease?

A. Upper-gastrointestinal x-ray

B. Gastric analysis

C. Endoscopy

D. Barium studies

79. Inflammation is a useful response which serves the following purposes except

A. Limit tissue injury

8. Localize and eliminate the causative agent

B. Respond to a particular cause

C. Restore tissue to normality or near normal

80. Which of the following type of surgery can be referred to as merely symptomatic operation?

A. Ablative surgery

B. Constructive surgery

C Exploratory surgery

C. Palliative surgery

81. The nurse working on a surgical unit would identify which client as having the highest risk for pulmonary complications after surgery?

A. A 24-year-old with open reduction internal fixation of the ulnar

B. A 45-year-old with an open cholecystectomy

C. A 36-year-old after a hysterectomy

D. A 50-year-old after a lumbar laminectomy

82. A client is admitted with benign prostatic hypertrophy. Which clinical manifestation should the nurse expect?

A. Frequent urge to void

B. Foul-smelling urine

C. Copious urine output

D. Pain on urination

A

83. The highest priority nursing care the nurse should give to a patient during the immediate postoperative period is by

- A. Checking the vital signs of every 15 minutes
- B. Maintaining a patent airway
- C. Observing for haemorrhage
- D. Recording fluid intake and output

84. Most peptic ulcers occurring in the stomach are in the.....

- A. Body of the stomach
- B. Cardiac portion
- C. Oesophageal junction
- D. Pyloric portion

85. Which of the following surgical consent would NOT be considered legal?

A. Consent signed by 21-year-old man who has a fractured left tibia due to an auto accident B. Consent signed by a 36-year-old lady one hour after receiving her preoperative medication

B. Consent signed by a 33-year-old manic man

C. Consent signed by a 60-year-old man the evening prior to surgery

86. A female client with hepatitis C develops liver failure and GI hemorrhage. The blood products that would most likely bring about hemostasis in the client are.

A. Whole blood and albumin.

B. Platelets and packed red blood cells.

C. Fresh frozen plasma and whole blood

D. Cryoprecipitate and fresh frozen plasma

87. The client returns to the recovery room following repair of an intrathoracic aneurysm. Which finding would require further investigation?

A. Pedal pulses bounding and regular

B. Urinary output 20ml. In the past hour

C Blood pressure 108/30

C. Oxygen saturation 97%

88. The nurse is doing bowel and bladder retraining for the client with paraplegia. Which of the following is not a factor for the nurse to consider?

A. Dietary patterns

B. Mobility

C. Fluid intake

D. Sexual

89. The client is admitted to the unit after a motor vehicle accident with a temperature of 102°F rectally. The nurse is aware that the most likely explanation for the elevated temperature is:

A. There was damage to the hypothalamus

B. He has an infection from the abrasions to the head and face

C. He will require a cooling blanket to decrease the temperature.

D. There was damage to the frontal lobe of the brain

90. The nurse is taking the blood pressure of an obese client. If the blood pressure cuff is too small, the results will be:

- A. A false elevation
- B. A false low reading

CA blood pressure reading that is correct

- C. A subnormal finding

91. A client with an abdominal aortic aneurysm is admitted in preparation for surgery. Which of the following should be reported to the doctor?

- A. An elevated white blood cell count
- B. An abdominal bruit
- C. A negative Babinski reflex
- D. Pupils that are equal and reactive to light

92. The doctor is preparing to remove chest tubes from the client's left chest. In preparation for the removal, the nurse should instruct the client to:

- A. Breathe normally
- B. Hold his breath and bear down
- C. Take a deep breath
- D. Sneeze on command

93. The nurse is taking the vital signs of the client admitted with cancer of the pancreas. The nurse is aware that the fifth vital sign is

- A. Temperature
- B. Pain
- C. Pulse
- D. Respiration

94. The doctor has ordered a patient-controlled analgesia (PCA) pump for the client with chronic pain. The client asks the nurse if he can become overdosed with pain medication using this machine. The nurse demonstrates understanding of the PCA if she states

- A. "The machine will administer only the amount that you need to control your pain without taking any action."

B. "The machine has a locking device that prevents overdosing to occur."

C. "The machine will administer one large dose every 4 hours to relieve your pain,

D. "The machine is set to deliver medication only if you need it."

95. A client has cancer of the pancreas. The nurse should be most concerned with which nursing diagnosis?

A. Alteration in nutrition

B. Alteration in bowel elimination

C. Alteration in skin integrity

D. Ineffective individual coping

96. The nurse is caring for a client with ascites. Which is the best method to use for determining early ascites?

A. Inspection of the abdomen for enlargement

B. Bimanual palpation for hepatomegaly

C. Daily measurement of abdominal girth

D. Assessment for a fluid wave

97. Which feline blood type is most common termed as the universal donor?

A. A

B. B

C. AB

D. O

98. Which feline blood type is most comunonly termed as the universal recipient?

A. A

B. B

C. AB

D. O

99. Nurse Kojo says his patient is blood group O Rhesus factor positive, what does the Rh factor mean

- A. Blood group is only O
- B. Patient tested positive for blood group
- C. A protein present in patient RBC
- D. None of the above

Below are the correct answers with brief rationales. I've kept the explanations exam-focused to help with licensure preparation.

1. Indications & Oxygen Therapy

1. D – Hypertension

Oxygen is not used to treat high blood pressure.

2. C – Monitoring oxygen saturation using a pulse oximeter

Ensures adequate oxygenation and patient safety.

3. B – Hypercapnia

Excess oxygen can suppress respiratory drive, especially in COPD.

4. C – Assessing the patient's respiratory status regularly

Airway and breathing are the top priorities.

5. A – To deliver a high concentration of oxygen

Non-rebreather masks deliver up to 95–100% oxygen.

6. B – Adjust the straps for a snug fit without discomfort

Ensures effective oxygen delivery and comfort.

7. C – Respiratory rate and effort

Airway and breathing are assessed first.

8. A – Every 2 hours

Patients on high-concentration oxygen need close monitoring.

2. Blood Transfusion

9. D – Obtaining informed consent

Must be done before starting transfusion.

10. B – Flushed skin and itching

Common allergic transfusion reaction signs.

11. A – Stop the transfusion immediately and notify the provider

This is the first and most critical action.

12. C – 4 hours

A unit of PRBCs must be transfused within 4 hours.

57. C – 1–1.5 g/dL

One unit of PRBC raises hemoglobin by ~1 g/dL.

58. B – Low blood pressure

Most common immediate donor risk.

59. D – None of the above

All listed are valid reasons for transfusion.

3. Bladder Catheterization

13. A – Urinary incontinence

Most common indication in clinical practice.

14. A – Using sterile technique

Prevents infection.

15. B – Urinary retention

Can occur due to blockage or bladder spasm.

16. A – Keep catheter bag below bladder level

Prevents backflow and infection.

64. D – Hold the catheter while the bladder empties

Prevents trauma and discomfort.

4. Obstetrics (APH & PPH)

17. D – Ectopic pregnancy

It is not antepartum hemorrhage (APH).

18. B – Monitoring fetal heart rate

Fetal wellbeing is the priority.

19. C – Left lateral position

Improves uteroplacental circulation.

20. C – Prepare for immediate cesarean section

Required in bleeding placenta previa.

21. A – Uterine atony

Most common cause of postpartum hemorrhage.

22. C – Massaging the uterus

First-line action for uterine atony.

23. C – Pale and clammy skin

Sign of hypovolemic shock.

5. Surgical Nursing & Instruments

24. D – All of the above

All reduce surgical site infection.

25. A – Supine

Standard position for abdominal surgery.

26. A – Supine

Used for head and neck surgeries.

27. C – Lithotomy

Used for anal and vulvar procedures.

28. A – Supine

Most thoracic surgeries.

29. B – Trendelenburg

Used for pelvic/lower abdominal surgery.

30. A – Dissecting forceps

Used to grasp tissues.

31. B – Scalpel

Used to cut tissues.

32. A – Forceps

Used during vaginal exams.

33. D – Artery forceps

Clamps blood vessels.

34. A – Surgeon scissors

Used to cut and dissect tissue.

35. B – Needle holder

Used to hold needles.

36. C – Suture material

Closes wounds.

37. D – Trocar

Creates entry in laparoscopic surgery.

38. A – Curette

Removes tissue or foreign material.

39. C – Hemostat

Manipulates delicate tissues.

40. A – Suture scissors

Removes sutures.

41. A – Uterine sound

Manipulates the uterus.

42. A – Snare

Removes polyps.

43. A – Dilator

Widens openings.

6. GI, Wounds & Medications

44. A – Constipation

Calcium carbonate commonly causes constipation.

45. A – BP 82/60, pulse 120

Signs of shock – report immediately.

46. C – Debridement

Removal of devitalized tissue.

47. B – Clean-contaminated wounds

Tracts entered under controlled conditions.

48. C – Tertiary healing

Delayed primary closure.

49. B – Proliferative phase

Granulation tissue and collagen formation.

50. A – Irregular-shaped lesion

Characteristic of malignant melanoma.

51. D – All of the above

Multiple sources of infection.

52. B – Ascorbic acid (Vitamin C)

Essential for collagen synthesis.

53. A – Cover wound with sterile towel & report

Emergency management of burst abdomen.

54. A – Increases effects of anesthesia

Kava-kava potentiates sedatives.

55. C – Using a heat lamp

Risk of burns – incorrect practice.

7. Post-Op, Cancer & Emergency

56. C – On the left side

Post-cholelithotomy positioning.

60. C – Avoid coke, tea, coffee

They worsen GERD.

61. B – Assist the client to dangle legs

Prevents orthostatic hypotension.

62. A – Ulcer perforation

Rigid board-like abdomen is classic.

63. D – Assess the pain first

Assessment precedes intervention.

65. A – To avoid working against gravity

66. B – Gastroesophageal reflux

Major risk factor for esophageal cancer.

67. D – Surgical-Medical Nursing

68. D – Palliative surgery

Relieves symptoms, not cure.

69. A – Elective surgery

70. A – Disseminated intravascular coagulation

71. A – Support arm with pillow when sitting

72. C – Sore throat

Expected with laryngeal radiation.

73. B – Splinting incision

Reduces pain during coughing.

74. D – Retinopathy of prematurity

75. A – Melena

Indicates GI bleeding.

76. D – BP 96/60, HR 120

Classic hemorrhagic shock signs.

77. D – Firm mass

Suggestive of malignancy.

78. C – Cyanotic ear lobes

Late sign of airway obstruction.

79. C – Endoscopy

Gold standard.

80. C – Respond to a particular cause

Inflammation is nonspecific.

81. D – Palliative surgery

82. B – Open cholecystectomy

High risk due to abdominal incision.

83. A – Frequent urge to void

84. B – Maintain a patent airway

85. D – Pyloric portion

86. B – Consent after pre-op medication

Client may not be mentally competent.

87. D – Cryoprecipitate & FFP

88. B – Urine output 20 ml/hr

Indicates poor perfusion.

89. D – Sexual

Not relevant to bowel/bladder retraining.

90. A – Damage to hypothalamus

91. A – False elevation

92. B – Abdominal bruit

Sign of aneurysm instability.

93. B – Hold breath and bear down

Prevents air entry.

94. B – Pain

Recognized as the fifth vital sign.

95. B – Locking device prevents overdose

96. A – Alteration in nutrition

97. C – Daily abdominal girth

8. Blood Groups

98. A – Blood type A (cats)

Most common donor type.

100. C – AB

Universal recipient.

101. C – A protein present on RBCs

Rhesus factor indicates presence of Rh antigen.

If you want, I can convert this into a printable revision table or mock exam format 